

NEWSLETTER

SUMMER 2003

DATA PROTECTION

ISSUES FOR

DISABILITY INSURERS

dispa
Disability Insurance
Service Providers Association

Introduction.

DISPA was formed a year ago with the purpose of improving and monitoring the professional standards of conduct and service of organisations providing services to disability insurers and to agree and publish standards of business principles, ethics and best practice. Members have noticed that the Data Protection Act 1998 is causing problems for both insurers and service providers and we decided that a Newsletter addressing some of the pertinent issues arising from this legislation and sharing our experiences would be useful.

This has been a difficult task. Data protection is a difficult subject as the law is still evolving and there is little case law to give us guidance. The Information Commissioners Office has given conflicting advice to DISPA members in what appear to be similar circumstances. Following correspondence with the Information Commissioners Office we still do not have firm guidelines that can be relied on and, as the Information Commissioner points out, their views can be challenged in the courts and might be overturned.

We have, therefore, sought to provide guidance on the initial view it is reasonable to take in given circumstances. We take the view that service providers with the exception of lawyers are Data Processors

and the correct line of any issues concerning the DPA should be through the insurers so that they retain control of the matter. A written agreement is clear evidence of the intent of both parties; the service provider can confirm the agreement in the acknowledgement letter of Insurer's instructions. DISPA members are happy to discuss this aspect with you, should you have any queries.



I hope that you will find this Newsletter both useful and interesting. We would appreciate your comments and to hear of any experiences you might have had involving the Data Protection Act 1998 that will help us to improve our understanding of this legislation. Please feel free to contact us at enquiries@dispa.org.uk

James A Harris, Chairman

COMMONLY ASKED QUESTIONS.

Subject Access Requests

1. Question:

If a service provider who has, say, carried out an independent medical examination is asked by the individual for a copy of the report, how should such a request be dealt with? In effect are service providers (other than lawyers) Data Processors or Data Controllers or both?

Answer:

This has been unclear due to conflicting comments from the Information Commissioner to two of our DISPA members. However, we interpret the distinction between Data Controllers and Data Processors to mean that service providers, with the exception of lawyers, are Data Processors rather than Data Controllers and as such the individual (the data subject) should re-direct the request to the Insurer who instructed the service provider. The service provider will provide information to its instructing Insurer. This also ensures that the Insurer is aware of all information being passed to the data subject in respect of their claim.

The DPA 1998 defines a Data Controller as a person(s) who determine the purposes for which, and the manner in which, personal data are, or are to be, processed. A Data Processor is any person(s) or persons (other than an employee of the Data Controller) who processes the data on behalf of the Data Controller. The DPA 1998 requires the Data Controllers to impose controls on the activities of the Data Processors they instruct by contract (Schedule 1, Part II, paras 11 and 12). Service providers (with the exception of lawyers) are Data Processors as they do not determine any purpose for which, or the manner in which, personal data are, or are to be, processed - it is the Insurer who does that.

2. Question:

Insurers have recently instructed Private Investigators for a second time to carry out

surveillance on a Claimant. The first surveillance exercise was carried out 6 months previously. Insurers have now received a Subject Access Request from the Claimant and the Insurer would prefer not to provide any information at this stage regarding the previous investigation or the one currently in progress, as to do so could prejudice the investigation process. How could this be approached?

Answer:

The Insurer should analyse the issue along the following lines; what was the reason for the first instruction for surveillance? It could be that the reason for initial instruction to the Private Investigators was because some type of 'criminal dishonesty' was suspected. If that was the case then, the first surveillance report and video could be protected by Section 29 of the DPA 1998. If the reason was because litigation was contemplated then it would be protected by legal professional privilege (as discussed in more depth below). Further it could be that the second instruction to the Private Investigator could be either protected by Section 29 of the DPA 1998 or Legal Professional Privilege even if the first one was not. Therefore, making a note on your file of the exact reasons why you instructed the Private Investigator may demonstrate the reason for protection from disclosure under the DPA 1998. Each set of circumstances is different, so it is advisable to seek specific legal advice if necessary.

Legal Professional Privilege

3. Question:

When is a document protected by Legal Privilege, and what effect does this have?

Answer:

If a document is protected by legal privilege that document (or the protected part) does

not need to be disclosed. Legal Professional privilege is one of the exemptions to the subject information provisions of the DPA 1998 (which includes the right of access to personal data). The questions to be asked to determine if a document or part of it is so privileged are:

- Was litigation contemplated when the document came into existence? If yes, the document can be protected by Legal Professional Privilege.
- If there are different reasons for a document coming into existence, what was the **dominant purpose** for that document being created? If the answer is to simply assess whether a claim should be paid (as opposed to, say, obtaining legal advice on whether or not it is a valid claim), it will not be privileged.
- Was the document written to or by a lawyer for the purposes of legal advice? If yes, whether or not litigation was contemplated, that document will be protected by Legal Professional Privilege.

The idea is that you must be able to consult your in-house or external lawyer in confidence. It is likely that the first medical report, which is obtained, has been produced to assess the claim and a dispute is not contemplated at that time. However, the circumstances of each case need to be considered. You may obtain a medical expert's report in a disputed claim, which you elect to disclose to the examined person (called waiving privilege) because the report clarifies why you have decided to turn down the claim.

Compliance with the DPA

4. Question:

An Insurer would like to instruct a Private Investigator to carry out surveillance on a particular Claimant. In order to comply with the DPA 1998, is it necessary to obtain any assurances from the Private Investigator

instructed with regards to how information is obtained or can the Insurers assume the Private Investigators know they are bound by the rules within the DPA 1998?

Answer:

As discussed above DISPA advocates as far as possible written agreements between service providers and Insurers should exist. It is important when instructing a Private Investigator to inform them that they are only being requested to operate within the confines of the DPA 1998 and they must also confirm that they are aware of and understand the DPA's provisions. This could also be achieved by Private Investigators ensuring the acknowledgement letter of receipt of instructions records these points. Section 55 of the DPA 1998 introduced a criminal offence for a person to knowingly or recklessly, without the consent of the Data Controller, obtain or disclose data or the information contained in personal data or procure the disclosure to another person of the information contained in personal data. The penalty is a fine of up to £5,000 in the Magistrate's Court or as unlimited fine in the Crown Court. If an Insurer authorises a solicitor to instruct a Private Investigator - who then procures personal data - both the Insurer and the solicitor may also be guilty of the criminal offence.

5. Question:

Can Insurers send copies of medical reports compiled by external medical examiners to Private Investigators when instructing them to help evaluate a suspicious claim?

Answer:

Much will depend upon the terms of the explicit consent, which the Claimant has given to the Insurer in respect of processing 'sensitive personal data'. Also the Insurer must think of the justification for passing on such information. Usually there would be proper justification and therefore a complaint about such information being passed should not be upheld. There is often information in these reports that can help identify the Claimant and provide more accurate information about their stated limitations.

Protection From Claimant's Comments on Medical Reports

6. Question:

A report has been obtained on a Claimant under AMRA (Access to Medical Reports Act) more than 6 months ago. At the time, the Claimant did not elect to see a copy of the report before it was sent to the Insurer. The Claimant has now sent the Insurer a Subject Access Request. Are the Insurers right to send that report direct to the Claimant?

Answer:

The Insurer should follow the Data Protection (Subject Access Modification) (Health) Order 2000. This includes an exemption against disclosure (similar to the exemption under AMRA). This means that if the supply of the medical report to the data subject would be likely to cause serious harm to the physical or mental health or condition of the data subject, then the Data Controller can withhold the report, or the relevant part of it; this is called the exemption test. The person who makes this decision is a health professional responsible for the clinical care of the data subject. Therefore, the Insurer should approach the author of the report first, sending him a copy of the report because he will not necessarily have kept a copy beyond the 6 months. The author of the report should be asked for a written opinion on whether the exemption test applies and whether (or to what extent) the medical report can be disclosed to the patient. If the person who supplied the report is not available, the next most suitable health professional can make the decision. It is feasible that the GP may defer the decision in a complex matter to the treating consultant. This consultation process does not apply when the data subject has already seen or knows about the information which is the subject of the request, and does not apply in certain limited circumstances where the consultation has been carried out prior to the request being made (article 7(1) and (2)). Sometimes an Insurer is aware that the treating doctors suspect a diagnosis of, say, MS but this has not been communicated to

the patient. This is an example of a situation, which the 2000 Order provides a process for dealing. After going through this process, the Insurer should consider whether there are any other applicable justifications for non-disclosure under the DPA 1998.

DISPA Members

Legal Services

Davies Lavery Solicitors
Rona Doyle & Co Solicitors
Kirbys Solicitors

Surveillance/Investigation Services

Devlin Associates Ltd
James Harris Investigations Ltd
GE Frankona

Medical Examination Services

Harley Street Claims
Medibureau Ltd
Williams Medico Legal Associates Ltd
PES

Visiting Services

GE Frankona
Health Claims Bureau Ltd

Absence Management

Medibureau Ltd
PES

Disability Assessment/Functional

Capacity Testing Services

PES
Personnel Assessment Testing

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www.dispa.org.uk for all contact names,
addresses and details

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